

Kinora Vascular System

Clinical AI Orientation (Part 17)

Arteries, Veins, Pulses, DVT/PE Screening & Compartment Syndrome

Version 1.0 — Kinora Admin Conocimientos Upload

Disclaimer

Educational only. Suspected DVT/PE, acute limb ischemia, or acute compartment syndrome — emergency medical care.

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1. Major Arteries (MSK-Relevant)

RECORD: Arterial Overview

ID: VASC-ART-01

Upper: Subclavian → axillary → brachial → radial/ulnar; branches to scapular anastomosis

Lower: Iliac → femoral → popliteal → anterior/posterior tibial & fibular → DP/PT pulses

Spinal: Vertebral arteries (cervical); radicular/Adamkiewicz (thoracolumbar cord risk)

References: Standring S. Gray's Anatomy. Moore KL et al. Clinically Oriented Anatomy. Netter FH. Neumann DA. Kinesiology of the Musculoskeletal System. Magee DJ. Orthopedic Physical Assessment. Brukner & Khan Clinical Sports Medicine.

2. Veins & DVT

RECORD: Venous / DVT Screening

ID: VASC-VEIN-01

Wells_DVT: Clinical prediction — high score → urgent medical pathway; do not rely on Homan's

Signs: Unilateral swelling, pain, erythema, warmth — refer for duplex

PE_Red_Flags: Dyspnea, chest pain, hemoptysis, syncope — emergency

References: Wells criteria literature; Standring S. Gray's Anatomy. Moore KL et al. Clinically Oriented Anatomy. Netter FH. Neumann DA. Kinesiology of the Musculoskeletal System. Magee DJ. Orthopedic Physical Assessment. Brukner & Khan Clinical Sports Medicine.

3. Pulse Assessment

RECORD: Pulses

ID: VASC-PULSE-01

UE: Brachial, radial, ulnar (Allen test)

LE: Femoral, popliteal, DP, PT

ABI: Ankle-brachial index for PAD suspicion

References: Standring S. Gray's Anatomy. Moore KL et al. Clinically Oriented Anatomy. Netter FH. Neumann DA. Kinesiology of the Musculoskeletal System. Magee DJ. Orthopedic Physical Assessment. Brukner & Khan Clinical Sports Medicine.

4. Compartment Syndrome

RECORD: Acute Compartment Syndrome

ID: VASC-CS-01

Signs: Pain out of proportion, pain on passive stretch, paresthesia — late: pulselessness

Action: Emergency fasciotomy pathway — do not elevate as sole care / delay

Chronic_Exertional: Reversible with rest — distinct from acute

References: Standring S. Gray's Anatomy. Moore KL et al. Clinically Oriented Anatomy. Netter FH. Neumann DA. Kinesiology of the Musculoskeletal System. Magee DJ. Orthopedic Physical Assessment. Brukner & Khan Clinical Sports Medicine.